

Chronic CSC and pachychoroid decisions

Part 2 | Localise the fluid and the choroidal phenotype

CSC is not synonymous with every subretinal fluid collection.

Finding	Implication
CSC	Serous detachment with characteristic leakage and choroidal changes
Pachychoroid spectrum	Shared choroidal phenotype, different clinical complications
Macular neovascularisation	Changes the treatment target
Steroid exposure	Review prescribed, topical and systemic routes

Before treating chronic fluid, establish what is leaking and whether there is a vascular network. A thick choroid is a clue rather than a complete diagnosis. The same OCT appearance may require very different treatment in CSC and neovascular disease.

VICI: a mechanistic story is not sufficient

Eplerenone did not outperform placebo for the trial visual endpoint.

Finding	Implication
Study design	Randomised, masked, placebo-controlled
Clinical setting	Treatment-naive chronic CSC
Primary finding	No superiority for visual acuity at twelve months
Practice implication	Do not present routine eplerenone as proven effective

This is a useful viva distinction: receptor biology can motivate a study without validating a treatment. Discuss the clinical endpoint and uncertainty rather than retaining a practice because its proposed mechanism sounds persuasive.

PLACE and SPECTRA: compare the actual alternatives

Half-dose PDT has supporting comparative evidence in chronic CSC.

Finding	Implication
PLACE	Half-dose PDT versus high-density micropulse
PLACE result	More complete fluid resolution with PDT
SPECTRA	Half-dose PDT versus oral eplerenone
SPECTRA result	Greater short-term fluid resolution with PDT

Neither trial proves that all laser methods, PDT settings or patient phenotypes are equivalent. A correct evidence answer names the comparator and acknowledges that anatomical drying, measured acuity and patient-reported function need not move together.

PDT: target the phenotype and understand the modification

A reduced protocol still requires lesion planning and consent.

Finding	Implication
Half-dose	Reduce photosensitiser exposure
Half-fluence	Reduce delivered light energy per area
Intended effect	Modify choroidal vascular leakage
Potential harm	Choroidal hypoperfusion and RPE changes

Describe the target, the modification and the endpoint. These three details make an intervention answer reviewable. Current verteporfin precautions and local availability should be checked independently before translating a study regimen into care.

Persistent symptoms: reassess what remains

Chronicity can leave damage after fluid has resolved.

Finding	Implication
Fluid persists	Reassess leakage pattern and neovascularisation
Fluid resolves, acuity poor	Review photoreceptor and RPE integrity
New haemorrhage	Investigate a secondary vascular lesion
Recurrence	Revisit exposures and imaging before retreatment

A difficult case should not end with the word persistent. State whether persistence refers to fluid, symptoms, a choroidal finding or structural injury. The next decision depends on that distinction and on the patient's functional priorities.